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Vaccine Hesitancy Writing Assignment

8 May 2022

Communication and Trust in Public Health: A Case Study of Vaccine Hesitancy in the United States During the 1976 Swine Flu Mass Immunization Program

Trust, or lack thereof, in public health institutions in the United States has characterized the reaction of the public to specific policies aimed to protect the population as a whole. Thus, it has been and continues to be vital for the government of the United States to promote trust in their health initiatives. When controversies surrounding these institutions arise, the public's trust in the government to safely and effectively implement policies is eroded. On April 13, 2022, the United States House of Representatives Committee on Oversight and Reform released a report that highlighted the role played by the consulting firm McKinsey and Company in the opioid epidemic (Maloney et al). The report found that several of the firm's consultants worked simultaneously on connected projects for pharmaceutical companies that produce opioids, such as Purdue Pharma, and the Food and Drug Administration (FDA), the government agency tasked with overseeing them (Maloney et al). At times, McKinsey and Company consultants used their work with the FDA in order to benefit their clients in the pharmaceutical industry by giving "opioid advice" to government officials (Maloney et al.). This report had significant ramifications as it came at a time when the United States continued to struggle with high rates of opioid misuse and abuse, leading to an increasing percentage of drug related deaths being tied to these highly addictive substances. In this context, the blatant conflict of interest present in the work of McKinsey and Company consultants whose clients included both pharmaceutical producers and the FDA prompted the distrust of the government's policies regarding opioids and the subsequent report spearheaded by Carolyn Maloney, the chair of the Committee on Oversight and Reform. McKinsey and Company's role in the opioid crisis in the United States highlights the importance of communication and trust in the realm of public health and the duty of the government to ensure that it upholds the necessary safety and ethical standards. This was not the first public health controversy in the United States that prompted mistrust and fear in its policies. In 1976, the

threat of a swine flu pandemic loomed over the United States. Remembering the horrors of past pandemics, public health officials quickly organized and instituted a mass swine flu immunization program in the United States. However, this program was swiftly derailed in large part due to significant vaccine hesitancy which was caused by ineffective communication on the necessity of vaccination and a breakdown of the public's trust in the United State government's health policies.

The possibility of a swine flu pandemic became a concern for public health officials after an outbreak of virus infections occurred at Fort Dix in New Jersey on January 27, 1976 (Dowdle S69). Several soldiers at the base were infected with a respiratory illness that was later determined to be caused by the swine flu virus (Dowdle S69-S70). One of those infected died (Dowdle S69). With memory of the 1918 influenza pandemic as well as the later flu outbreaks of 1957 and 1968, public health officials worried about another iteration of the widespread transmission of the deadly disease. The 1968 influenza outbreak infected around one in five Americans and resulted in roughly 33,000 deaths (Rovrik 4). Acting swiftly, the Centers for Disease Control (CDC) established nationwide surveillance on February 14, and on March 13, the CDC recommended a mass immunization program (Dowdle S69-S71). The CDC's recommendation was based on the conclusion of the Advisory Committee on Immunization Practices (ACIP) that a swine flu pandemic was possible (Dowdle S69-S71). However, the ACIP was uncertain over the likelihood that it would occur (Dowdle S69-S71). On March 24, in a meeting that included President Gerald Ford, and representatives of the CDC, the Food and Drug Administration (FDA), and the National Institute of Health (NIH), the decision to implement a mass vaccination program was adopted (Dowdle S69-S71). According to Walter R. Dowdle in his article "The 1976 Experience" in the *Journal of Infectious Diseases*, the push for mass immunization for a disease that was uncertain to cause a pandemic was justified by public health officials on the idea that it was better to "gamble with dollars than lives" (Dowdle S69-S71). Dr. Harvey V. Fineberg, former president of the National Academy of Medicine, argued that the response to the swine flu was interpreted by public health officials as an all-or-nothing scenario (Fineberg 414-415). In less than two months since the discovery of the virus at Fort Dix, the United States had incorporated a mass immunization program into its public health response in "one fell

swoop” according to Fineberg while the uncertainty of the threats posed by the disease to the public still loomed (Fineberg 414-415). Public health officials wanted to act swiftly and embark on vaccination to prevent not just a pandemic, but any outbreak of swine flu. However, the push to act as quickly as possible led to decisions being made without the full understanding of the swine flu or the vaccine developed to counteract it.

Hesitancy to the swine flu immunization program began with the government’s ineffective communication on the necessity of the vaccination. Despite the Ford Administration’s decision to go all-in on mass immunization in March, logistical constraints including producing and establishing effective distribution methods prevented the start of inoculations for several months (Dowdle S69-S71). In the ensuing months, there were no documented swine flu outbreaks in the United States according to Dowdle (Dowdle S69-S71). The earlier uncertainty of the ACIP on the possibility of a swine flu pandemic paired with the lack of epidemics led many leading medical experts and the public to doubt the vaccine’s necessity.

In the article “The Great Bicentennial Medical Experiment ” in the publication *The Great Speckled Bird*, David M. Rovrik elucidates the public’s growing skepticism towards the swine flu vaccine. The public’s hesitancy towards the swine flu vaccine was based on the absence of scientific evidence on the threat of the virus and the lack of support from the medical community for mass immunization. Dr. Martin Goldfield, a doctor who studied the Fort Dix outbreak who Rovrik cited throughout his article, noted that a different strain of influenza, “A-Victoria”, was more dominant than the swine flu strain as it was responsible for the majority of the cases (Rovrik 4-5). Rovrik cited another leading medical researcher, Dr. Sidney Wolfe, who argued that “modern antibiotics” would prevent the scale and severity of any outbreak of influenza, including swine flu (Rovrik 4-5). Rovrik, like a significant percentage of the population, believed that there was “no hard proof” that swine flu posed a significant medical threat, and thus there was no need for a vaccine (Rovrik 4-5). Furthermore, he noted that the mass swine flu immunization program was not backed by “science advisors” at the CDC or Dr. J. Anthony Morris, a researcher at the Division of Biological Standards (DBS), a government organization that evaluates

vaccine safety and efficacy (Rovrik 4-6). The hesitancy towards the swine flu vaccine displayed by Rovrik, and the broader United States public, was a result of skepticism over the likelihood of a swine flu pandemic and the lack of unanimous support from the medical community for the mass immunization program, preventing the government from effectively persuading people to be inoculated.

The hesitancy towards the swine flu vaccine was increased due to a breakdown in the people's trust of the government's public health policies, leading to an end of the mass immunization program. At the time there was another factor in play, which was growing concern over the motives of large and wealthy pharmaceutical companies and their significant lobbying power over government agencies. In his worry over this trend, Rovrik identified the findings of the 1971 Ribicoff report, an investigation of the DBS led by Senator Abraham Ribicoff (Rovrik 4-5). The report found that the agency was "too closely related" with pharmaceutical companies and often prioritized the sale of vaccines over their efficacy and safety (Rovrik 4-5). The report found numerous instances of the DBS ignoring recommendations of scientists and covering up research that would be detrimental to the sale of vaccines, even despite legitimate health concerns (Rovrik 4-5). In short, the evaluation of the safety and efficacy of the vaccine was performed by those "whose business it is to sell" the immunization (Rovrik 4-5). The findings of the Ribicoff report led many, including Rovrik, to question whether the mass immunization was conducted in the interests of protecting public health, or was merely a corrupt attempt to generate revenue through the sale of unnecessary vaccines. Thus, the public's growing distrust of the government's public health agenda due to the influence of the pharmaceutical industry over government agencies led many to become suspicious of the safety and efficacy of the vaccines.

Another factor that contributed to the public's growing suspicion of the safety and efficacy of the swine flu vaccine was caused by past and present failures to effectively ensure the quality of medical products. For example, in 1955, during the polio immunization program, the failure of one manufacturer to inactivate the poliomyelitis virus in the vaccine led to many cases of the disease (Rovrik 4-5). Of these cases, 192 were classified as paralytic polio and ten died (Rovrik 4-5). According to the Ribicoff report, the incident was an example of the DBS failing to adequately follow the recommendations of scientists

who were concerned about the process that the manufacturer was using to inactivate the virus (Rovrik 4-5). In addition, the virus SV-40 was a contaminant that was found and removed from the polio immunization, but only after millions of vaccines had been distributed (Rovrik 4-5). In 1976, the long term consequences of this contamination were still unknown, though there were significant health concerns (Rovrik 4-5). The failure of the DBS to effectively uphold safety and efficacy standards for the vaccine was fresh in the minds of many during the 1976 swine flu immunization campaign. Rovrik cites the incident as the reasoning for his skepticism of the swine flu vaccine and his desire to see extensive testing for “dangerous side effects” before the mass immunization campaign (Rovrik 4-5). With the past failures of the government to enact its public health agenda safely and effectively, many were concerned about the dangers of the mass immunization program during the swine flu hysteria.

The public’s fear of the dangers of the mass immunization program were realized with an increase in cases of Guillain-Barre Syndrome (GBS) that were tied to the vaccine. The increase in GBS was first reported in Minnesota in November 1976, and after studying the disease’s connection with vaccination, the CDC suspended the mass swine flu immunization program on December 16 (Dowdle S70-S72). The failure of the government, specifically the health agencies such as the DBS, to ensure the safety of the vaccine was in part due to the lack of time to conduct extensive studies into the vaccine before its distribution to the public (Dowdle S70-S72). Because of the vaccine’s lack of testing, Rovrik labeled the vaccination program as a “medical experiment” that used United States residents as the “test subjects” (Rovrik 4-6). To the public, who were already worried that the government’s public health agenda was corrupted by its desire to sell vaccines due to its relationship with the pharmaceutical industry, the association of the swine flu immunization with GBS led to a complete breakdown in trust. Combined with the lack of effective communication regarding the necessity of the vaccine as there was sparse scientific evidence that swine flu posed a significant public health threat, the erosion in the trust of the government’s public health agenda led people to avoid vaccination as it was seen as dangerous and an attempt to generate revenue for the pharmaceutical industry rather than protect the health of the people.

While it is important to limit disease transmission and prevent future pandemics if at all possible, the public health response to the swine flu failed to effectively and efficiently respond to the threat posed by the virus. In acting swiftly and without allowing for studies and tests to understand the swine flu virus or the safety and efficacy of the vaccine, the government damaged its credibility in regards to public health measures. Trust and communication play a significant role in enacting successful public health policy, and one stark example of this is the mass swine flu immunization campaign of 1976. The failure of the government to effectively communicate the need for immunization as well as a breakdown of the people's trust of the public health agenda led to significant vaccine hesitancy, ultimately leading to the end of the mass vaccination campaign.